

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Emtricitabine/Tenofovir disoproxil 200mg/245mg tablets (Truvada) for the treatment of HIV Pre-Exposure Prophylaxis (PrEP)

Summary of NICE / NICE CKS Guidelines for HIV Pre-Exposure Prophylaxis (PrEP)

Overview

Pre-exposure prophylaxis (PrEP) is an antiretroviral medication taken by HIV-negative individuals at high risk of HIV acquisition. Truvada is a combination of emtricitabine and tenofovir disoproxil, indicated for prevention of HIV-1 in high-risk populations.

Assessment

- **HIV test:** Must be negative (4th generation test within 7 days)
- **Hepatitis B status:** Must be documented; co-infection requires specialist co-management
- **Renal function:** eGFR must be ≥ 60 mL/min/1.73m²
- **Sexual history & risk factors:** Comprehensive assessment for high-risk populations (MSM, heterosexual with HIV-positive partner, sex workers, PWID)
- **STI screening:** Baseline screening for other sexually transmitted infections

Management

- **Daily PrEP:** One tablet once daily
- **Event-based (MSM only):** 2 tablets 2-24 hours before sex, 1 tablet 24 hours after first dose, 1 tablet 48 hours after first dose
- **Follow-up:** 3-monthly review including HIV test, renal function assessment, and STI screening

Red flags

- Symptoms of acute HIV seroconversion (fever, rash, arthralgia, myalgia)
- Renal impairment (eGFR <60)
- Hepatitis B co-infection without specialist co-management
- Suspected bone mineral density loss (long-term tenofovir users)

Patient Group Direction

for the supply/administration of

Emtricitabine/Tenofovir disoproxil 200mg/245mg tablets (Truvada)

for the treatment of

HIV Pre-Exposure Prophylaxis (PrEP)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Prevention of HIV-1 transmission in HIV-negative individuals assessed at high risk of acquisition (men who have sex with men, heterosexual individuals with HIV-positive partners, sex workers, people who inject drugs).
Inclusion criteria	Adults aged 18 years or over Confirmed HIV-negative status (4th generation HIV test within 7 days of starting PrEP) eGFR ≥ 60 mL/min/1.73m² Assessed as high risk for HIV acquisition Capacity to provide informed consent Hepatitis B status documented (if positive, must be under specialist co-management)
Exclusion criteria	HIV-positive or unknown HIV status eGFR < 60 mL/min/1.73m² Hepatitis B co-infection without specialist co-management Concurrent nephrotoxic medications (e.g., NSAIDs, cisplatin, amphotericin B) Known hypersensitivity to emtricitabine, tenofovir, or excipients

	Suspected acute HIV seroconversion symptoms
Cautions	Renal impairment: Monitor renal function 3-monthly; dose adjustment required if eGFR 30-59 Bone mineral density: Long-term tenofovir use associated with BMD loss; consider DEXA scanning in long-term users Hepatitis B: Risk of hepatitis flare on discontinuation; ensure prophylaxis if applicable Lactic acidosis: Rare but serious; monitor for symptoms (fatigue, dyspnoea, hypoxia) Drug interactions: Ritonavir, ritonavir-boosted protease inhibitors, and some anticonvulsants may alter levels Mitochondrial toxicity: Monitor for symptoms of mitochondrial dysfunction
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Emtricitabine 200mg / Tenofovir disoproxil 245mg tablets
Legal category	POM
Storage	Store below 30°C. Keep in a dry place.
Route/method of administration	Oral
Dose and frequency	Daily regimen: 1 tablet once daily Event-based (MSM only): 2 tablets 2-24 hours before sexual activity, 1 tablet 24 hours after first dose, 1 tablet 48 hours after first dose
Quantity to be administered and/or supplied	Up to 30 tablets (30-day supply for daily dosing)
Maximum or minimum treatment period	Ongoing with 3-monthly review. Requires 3-monthly HIV test, renal function (eGFR), and STI screening.
Adverse effects	Common ($\geq 1/100$): Nausea, diarrhoea, headache, dizziness, fatigue, rash Uncommon ($\geq 1/1000$): Renal tubular dysfunction, decreased bone mineral density, hepatic steatosis, abdominal pain, vomiting Rare ($\geq 1/10,000$): Fanconi syndrome, lactic acidosis, pancreatitis, Stevens-Johnson syndrome, toxic epidermal necrolysis

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Provide the patient information leaflet (PIL) supplied with Truvada. Discuss adherence importance for daily dosing or correct timing for event-based dosing.
Follow-up advice to be given to patient or carer	Take the medication exactly as prescribed (daily or event-based as agreed) For daily dosing, aim to take at the same time each day For event-based dosing, ensure correct timing: 2 tablets 2-24 hours before sex, 1 tablet 24 hours later, 1 tablet 48 hours later Attend 3-monthly follow-up appointments for HIV testing, renal function checks, and STI screening Seek immediate medical attention if symptoms of acute HIV infection develop (fever, rash, muscle/joint pain, night sweats) PrEP does not provide protection against other sexually transmitted infections; use condoms If a dose is missed, take it as soon as remembered unless close to the next dose; do not double dose Report any symptoms of renal impairment (polyuria, polydipsia) to your healthcare provider Avoid NSAIDs and other nephrotoxic drugs without medical advice

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Pre-Exposure Prophylaxis (PrEP) for HIV-1 Prevention
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025